Taipei Veterans General Hospital

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Sample Information

Patient Name: 余林瑞菊
Gender: Female
ID No.: S202186725
History No.: 14143772
Age: 83

Ordering Doctor: DOC1878G 沈佳儀
Ordering REQ.: 0CNAVWW
Signing in Date: 2023/07/13

Path No.: M112-00176
MP No.: F23055
Assay: Oncomine Focus Assay
Sample Type: FFPE
Block No.: S112-28074D
Percentage of tumor cells: 40%

Reporting Doctor: DOC5466K 葉奕成 (Phone: 8#5466)

Note:

Sample Cancer Type: Non-Small Cell Lung Cancer

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Report Highlights

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Relevant Non-Small Cell Lung Cancer Variants

Gene	Finding	Gene	Finding
ALK	None detected	NTRK1	None detected
BRAF	None detected	NTRK2	None detected
EGFR	None detected	NTRK3	None detected
ERBB2	None detected	RET	None detected
KRAS	None detected	ROS1	None detected
MET	***MET amplification***		

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.06(005). The content of this report has not been evaluated or approved by FDA, EMA or other regulatory agencies.

Relevant Biomarkers

Tier	Genomic Alteration	Relevant Therapies (In this cancer type)	Relevant Therapies (In other cancer type)	Clinical Trials
IA	*MET amplification* MET proto-oncogene, receptor tyrosine kinase	capmatinib crizotinib tepotinib	None	2

Public data sources included in relevant therapies: FDA[1], NCCN, **EMA**[2], ESMO
Tier Reference: Li et al. *Standards and Guidelines for the Interpretation and Reporting of Sequence Variants in Cancer: A Joint Consensus Recommendation of the Association for Molecular Pathology, American Society of Clinical Oncology, and College of American Pathologists.* J Mol Diagn. 2017 Jan;19(1):4-23.

Variant Details

DNA Sequence Variants

Gene	Amino Acid Change	Coding	Variant ID	Locus	Allele Frequency	Transcript	Variant Effect	Coverage
JAK1	p.(P733=)	c.2199A>G	.	chr1:65310489	42.36%	NM_002227.4	synonymous	1997
ALK	p.(D1529E)	c.4587C>G	.	chr2:29416366	30.12%	NM_004304.5	missense	1999
ALK	p.(A1200=)	c.3600G>C	.	chr2:29443617	29.65%	NM_004304.5	synonymous	1973
FGFR4	p.(P136L)	c.407C>T	.	chr5:176517797	99.30%	NM_213647.3	missense	2000
JAK3	p.(L485=)	c.1453C>T	.	chr19:17949188	52.10%	NM_000215.4	synonymous	2000

Copy Number Variations

Gene	Locus	Copy Number
MET	chr7:116313480	7.15

Biomarker Descriptions

MET (MET proto-oncogene, receptor tyrosine kinase)

Background: The MET proto-oncogene encodes a receptor tyrosine kinase for the hepatocyte growth factor (HGF) protein, which is expressed by mesenchymal cells. MET is expressed as multiple isoforms with transcript variant 1 (NM_001127500.3) encoding a 1408 amino acid protein and transcript variant 2 (NM_000245.4) encoding a 1390 amino acid protein, both of which possess an intact protein kinase domain[1]. Ubiquitin-dependent proteolysis is responsible for regulating the steady state level of the MET protein via recognition of the tyrosine phosphorylation site Y1003(NM_000245.4), sometimes referred to as Y1021 (NM_001127500.3), in the MET Cbl-binding domain within the juxtamembrane region[2,3,4]. Growth factor signaling leads to MET dimerization and subsequent initiation of downstream effectors including those involved in the RAS/RAF/MEK/ERK and PI3K/AKT signaling pathways, which regulate cell migration, proliferation, and survival[5,6].

Alterations and prevalence: Somatic mutations in MET are observed in 10% of uterine corpus endometrial carcinoma, 9% of skin cutaneous melanoma, 8% of papillary renal cell carcinoma (PRCC), and 4% of lung adenocarcinoma, colorectal adenocarcinoma, bladder urothelial carcinoma, and uterine carcinosarcoma[7,8]. Recurrent somatic MET alterations include activating mutations, gene amplification, and translocations generating MET gene fusions. Recurrent somatic mutations fall into two classes, mutations in the MET kinase domain, which are uncommon, and splice-site mutations affecting exon 14. Recurrent kinase domain mutations are observed in PRCC and include M1250T, H1094Y, and V1070E (NM_000245.4)[7,8]. Mutation of the Y1003 phosphorylation site is reported in approximately 2% of MET altered lung cancer[9]. In contrast, splice-site mutations flanking exon 14 are observed in 3-4% of all non-small cell lung cancer (NSCLC)[10]. These mutations include canonical splice site mutations affecting exon 14 and deletions that extend into the splicing motifs within intron 13[9,11]. Such mutations disrupt splicing leading to the formation of an alternative transcript that joins exon 13 directly to exon 15 and skips exon 14 entirely. The MET exon 14 skipping transcript lacks the juxtamembrane domain that contains the recognition motif for ubiquitin-dependent proteolysis and thus leads to a marked increase in the steady-state level of the MET protein[12]. MET exon 14 skipping mutations act as oncogenic drivers in lung cancer mutually exclusive to activating mutations in EGFR and KRAS and other oncogenic fusions such as ALK and ROS1[11,13,14]. MET is amplified in 2-5% of ovarian cancer, esophageal adenocarcinoma, stomach adenocarcinoma, glioblastoma, and lung adenocarcinoma[7,15,16]. Recurrent MET fusions, although infrequent, are observed in adult and pediatric glioblastoma, papillary renal cell carcinoma, lung cancer, liver cancer, thyroid

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Biomarker Descriptions (continued)

cancer, and melanoma[17,18,19]. MET alterations are believed to be enriched in late-stage cancers where they drive tumor progression and metastasis[20,21,22].

Potential relevance: In 2020, the FDA granted accelerated approval to capmatinib[23] for NSCLC harboring MET exon 14 skipping positive as detected by an FDA-approved test. The kinase inhibitor, tepotinib[24], is also approved (2021) for MET exon 14 skipping mutations in NSCLC. MET exon 14 skipping mutations confer sensitivity to approved kinase inhibitors including crizotinib (2011), which is recommended for MET amplifications and exon 14 skipping mutations[11,13,14,25]. The FDA also granted breakthrough therapy designation (2018) to crizotinib for metastatic non-small cell lung cancer (NSCLC) with MET exon 14 alterations with disease progression on or after platinum-based chemotherapy[26]. Conversely, amplification of MET has been observed to mediate resistance to EGFR tyrosine kinase inhibitors (TKIs)[27,28,29,30,31]. However, the FDA has granted Fast Track designation (2021) to the MET/CSF1R/SRC small molecule inhibitor, TPX-0022[32], for MET amplified advanced or metastatic gastric cancer, including gastroesophageal junction adenocarcinoma (GEJ) after prior chemotherapy. Tepotinib has also been recommended for treatment of NSCLC with high-level MET amplification[25]. In a phase II trial testing the MET inhibitor savolitinib, patients with advanced PRCC exhibited median progression free survival (PFS) of 6.2 and 1.4 months for MET-driven and MET-independent PRCC, respectively[33].

Relevant Therapy Summary

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types ✖ No evidence

MET amplification

Relevant Therapy	FDA	NCCN	EMA	ESMO	Clinical Trials*
capmatinib	✖	●	✖	✖	✖
crizotinib	✖	●	✖	✖	✖
tepotinib	✖	●	✖	✖	✖
bozitinib	✖	✖	✖	✖	● (I/II)
HLX55	✖	✖	✖	✖	● (I)

* Most advanced phase (IV, III, II/III, II, I/II, I) is shown and multiple clinical trials may be available.

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Relevant Therapy Details

Current NCCN Information

● In this cancer type ○ In other cancer type ◑ In this cancer type and other cancer types

NCCN information is current as of 2023-05-01. For the most up-to-date information, search www.nccn.org.
For NCCN International Adaptations & Translations, search www.nccn.org/global/what-we-do/international-adaptations.

Some variant specific evidence in this report may be associated with a broader set of alterations from the NCCN Guidelines. Specific variants listed in this report were sourced from approved therapies or scientific literature. These therapeutic options are appropriate for certain population segments with cancer. Refer to the NCCN Guidelines® for full recommendation.

MET amplification

capmatinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** MET amplification

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Metastatic (Line of therapy not specified)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

crizotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** MET amplification

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Metastatic (Line of therapy not specified)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

tepotinib

Cancer type: Non-Small Cell Lung Cancer **Variant class:** MET amplification

NCCN Recommendation category: 2A

Population segment (Line of therapy):
- Metastatic (Line of therapy not specified)

Reference: NCCN Guidelines® - NCCN-Non-Small Cell Lung Cancer [Version 3.2023]

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Clinical Trials in Taiwan region:

Clinical Trials Summary

MET amplification

NCT ID	Title	Phase
NCT03175224	Phase I/II Multicenter Study of the Safety, Pharmacokinetics, and Preliminary Efficacy of APL-101 in Subjects With Non-Small Cell Lung Cancer With c-Met EXON 14 Skip Mutations and c-Met Dysregulation Advanced Solid Tumors.	I/II
NCT04169178	A Phase I Dose Finding/Expansion Study of HLX55, A Monoclonal Antibody Targeting Tyrosine-Protein Kinase MET (C-MET) in Patients With Advanced Solide Tumors Refactory to Standard Therapy	I

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